

Pembrolizumab + carboplatin/paclitaxel -> AC

Breast - NCCP 00857 - version 3

Your medicines

Pembrolizumab - Carboplatin - Paclitaxel - Doxorubicin - Cyclophosphamide

Your treatment schedule

Two neoadjuvant 21-day phases, then surgery and adjuvant pembrolizumab.

A picture of side effects

Tell your team about new or worsening symptoms. Numbers link each symptom box to the body map.

Pembrolizumab - immune effects

Chemotherapy - phase-specific effects

1 Hormones / brain

Pembrolizumab

Unusual fatigue, dizziness, headache or thirst should be reported.

3 Heart

Doxorubicin / pembrolizumab

New chest symptoms, palpitations or breathlessness need assessment.

5 Nerves

Paclitaxel

Report tingling or numbness before it persists or affects everyday activity.

7 Skin / nails

Chemotherapy / pembrolizumab

Rash, nail changes or an infusion reaction can occur. Severe or blistering rash needs urgent review.

2 Lungs

Pembrolizumab

Report new cough or breathlessness promptly.

4 Liver / kidneys

Pembrolizumab

Blood tests monitor organ function. Report yellow eyes, dark urine or reduced urine.

6 Mouth / bowel

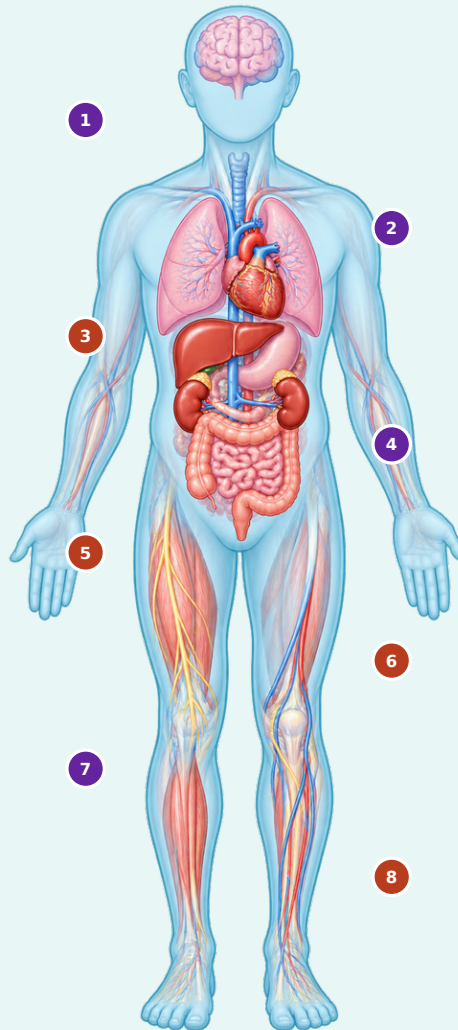
Chemotherapy / pembrolizumab

Report mouth soreness, persistent diarrhoea, vomiting or severe tummy pain.

8 Blood / infection

Chemotherapy

Fever or sudden illness needs immediate oncology contact. Medicines change between treatment phases.



Selected effects only. The map does not show likelihood and cannot diagnose the cause of a symptom.

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Your treatment journey

Cycles 1-4: Every 21 days: pembrolizumab + carboplatin on day 1; paclitaxel on days 1, 8 and 15.

Cycles 5-8: Every 21 days: pembrolizumab + doxorubicin + cyclophosphamide on day 1.

Surgery: Definitive breast surgery follows the neoadjuvant phase when clinically appropriate.

Adjuvant phase: Pembrolizumab continues after surgery: 200 mg every 21 days for 9 cycles or 400 mg every 42 days for 5 cycles, according to the treatment plan.

Throughout: Bloods and toxicity review guide whether treatment is given, delayed or modified.

Monitoring and support

- Blood count, kidney/liver profile and glucose are checked regularly; thyroid testing is repeated during pembrolizumab treatment.
- Paclitaxel requires premedication according to the regimen/local protocol.
- Cardiac assessment may be required before the doxorubicin phase.
- Your current medicines, including non-prescription products.
- A note of new symptoms, when they started and what they stopped you doing.
- Any temperature, blood-pressure or bowel-count readings your team asked you to record.
- Questions about the purpose of treatment, scan/review timing or changes to the plan.

WHEN TO CALL

Call oncology urgently for fever, new breathlessness, persistent diarrhoea, jaundice, severe rash, significant bleeding, severe weakness or a new neurological symptom. For severe breathing difficulty, chest pain, collapse or seizure call 112/999. If your team has given you different local thresholds or emergency instructions, follow those instructions.

Consent discussion support

Use the approved consent process to document treatment intent, expected benefit, material risks, alternatives, questions and the patient's decision.

Scan to open the regimen-specific patient portal

<https://sactcheck.com/patient/00857/>

Official NCCP 00857 v3

SACTCheck v0.80.4 - patient information prototype for clinical review. Follow your oncology team and local emergency instructions.

